SERIOUS vs NON-SERIOUS CASE ASSESSMENT

Pharmacovigilance Internship – Severity & Case Classification

Objective: Review selected patient case scenarios, assess regulatory seriousness using the ICH E2A seriousness concepts, distinguish seriousness from expectedness, and document the appropriate expedited-reporting assessment.

Assessment approach

Seriousness and severity are not treated as the same concept. The supplied case data and clinical outcome were reviewed against the seriousness criteria. Expectedness was considered separately when determining whether an expedited reporting timeline applies. Facts not provided in the case pack were not inferred.

Classification matrix

Case ID	Suspect product	Reaction	Classification	Expectedness	Reporting assessment
SYN-PV-005	Ciprofloxacin	Tendon pain	Serious	Known	No expedited reporting based on expectedness; routine reporting per applicable requirements
SYN-PV-008	Losartan	Fatigue	Non-Serious	Unexpected	No expedited reporting; routine reporting per applicable requirements
SYN-PV-010	Atorvastatin	Myalgia	Serious	Known	No expedited reporting based on expectedness; routine reporting per applicable requirements
SYN-PV-019	Metformin	Nausea	Non-Serious	Known	No expedited reporting; routine reporting per applicable requirements
SYN-PV-024	Losartan	Fatigue	Serious	Unexpected	15 calendar days – expedited reporting

Individual case assessments

SYN-PV-005 – Ciprofloxacin / Tendon pain

Patient	33 years; Sex: Unknown
Indication	Infection
Dose / route	500 mg twice daily; Oral
Onset	15 day(s) after treatment began
Outcome	treatment was stopped and the patient was not recovered at follow-up
Seriousness	Serious – Medically important
Expectedness	Known
Dechallenge / Rechallenge	Unknown / Not done
Assessment rationale	The case is recorded as medically important; this supports Serious classification under the medically-important-event criterion.
Reporting assessment	No expedited reporting based on expectedness; routine reporting per applicable requirements

SYN-PV-008 – Losartan / Fatigue

Patient	36 years; Sex: Male
Indication	Infection
Dose / route	5 mg once daily; Oral
Onset	2 day(s) after treatment began
Outcome	treatment was stopped and the patient was recovering
Seriousness	Non-Serious – Not serious
Expectedness	Unexpected

Dechallenge / Rechallenge	Negative / Not done
Assessment rationale	The supplied case data identifies the event as Not serious.
Reporting assessment	No expedited reporting; routine reporting per applicable requirements

SYN-PV-010 – Atorvastatin / Myalgia

Patient	57 years; Sex: Female
Indication	Infection
Dose / route	5 mg once daily; Oral
Onset	21 day(s) after treatment began
Outcome	treatment was stopped and the patient was recovered
Seriousness	Serious – Hospitalisation
Expectedness	Known
Dechallenge / Rechallenge	Unknown / Negative
Assessment rationale	Hospitalisation is a regulatory seriousness criterion; therefore the case is Serious.
Reporting assessment	No expedited reporting based on expectedness; routine reporting per applicable requirements

SYN-PV-019 – Metformin / Nausea

Patient	29 years; Sex: Unknown
Indication	Anxiety
Dose / route	5 mg once daily; Oral
Onset	9 day(s) after treatment began
Outcome	treatment was stopped and the patient was recovering
Seriousness	Non-Serious – Not serious
Expectedness	Known
Dechallenge / Rechallenge	Positive / Negative
Assessment rationale	The supplied case data identifies the event as Not serious.
Reporting assessment	No expedited reporting; routine reporting per applicable requirements

SYN-PV-024 – Losartan / Fatigue

Patient	79 years; Sex: Unknown
Indication	Hypertension
Dose / route	500 mg twice daily; Oral
Onset	3 day(s) after treatment began
Outcome	treatment was stopped and the patient was recovering
Seriousness	Serious – Hospitalisation
Expectedness	Unexpected
Dechallenge / Rechallenge	Positive / Positive
Assessment rationale	Hospitalisation is a regulatory seriousness criterion; therefore the case is Serious.
Reporting assessment	15 calendar days – expedited reporting

Key regulatory notes

1. Seriousness is a regulatory concept. Examples include death, life-threatening events, hospitalisation or prolongation of hospitalisation, persistent/significant disability or incapacity, congenital anomaly/birth defect, and other medically important events.

2. Expedited reporting is generally linked to seriousness and unexpectedness. For a serious and unexpected suspected adverse reaction, the standard expedited reporting target used in this exercise is 15 calendar days. A serious but expected event is not automatically expedited solely because it is serious.

3. Non-serious cases do not meet the expedited-reporting threshold in this assessment and should be handled through routine reporting/recording according to the applicable procedure and reporting rules.

4. This report is based only on the fictional/de-identified Workora training data supplied in the classification pack. It is an educational exercise and is not a clinical or regulatory submission.

Prepared for: PV Internship Submission

Prepared by: Prathamesh Vijay Nikum